
PREPARED FOR CARBON

Hospital receivables, recovered and advanced

A partnership in which Carbon funds and we vet, reconcile and collect.

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DATE

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Private hospitals in Nigeria are owed money they have already earned.

Most of a private hospital's receivables are owed by health plans, with corporate accounts and the national scheme behind them. Payment commonly takes three to six months, and some claims are never settled at all because nobody on either side has the time to work through the dispute. Meanwhile salaries, consumables and the pharmacy are paid this month.

We see this from inside. In one hospital we audited this year, about a month of revenue was sitting outside the business, split between unpaid claims and stock. Its owners thought it had a profit problem. It was short of cash, which needs a different fix.

A hospital that is trading at a profit can still run out of cash, and unpaid claims are usually the reason.



Most of it is disputed. Very little of it is refused.

Why the claim is unpaid	What clears it	Recoverable
No pre-authorisation code, or the code does not match the service billed	Retrospective authorisation, agreed with the plan's medical team	Usually
Billed above the agreed tariff, or on an old tariff	Rebill at the agreed rate; the gap is written off, not the claim	Mostly
Missing documents: notes, results, discharge summary	Pull the record and resubmit inside the plan's window	Usually
Submitted late, or never submitted	Depends on the plan's window and goodwill	Sometimes
The plan is slow to pay across the board	Escalation and a payment schedule	Slowly
The enrollee was not covered, or the plan has failed	Bill the patient or employer, or write it off	Rarely

Recovery here is mostly reconciliation. We know the people in the claims and medical teams at the plans, and a dispute settled at that level gets paid without anyone being threatened.



Vet, reconcile, recover

One desk, run by people who have run hospitals and sat across the table from health plans.

Vet

Every claim is checked against the plan's tariff, the authorisation record and the clinical notes before it is funded. Claims that will not pay are kept out of the advance and sent to repair instead.

Reconcile

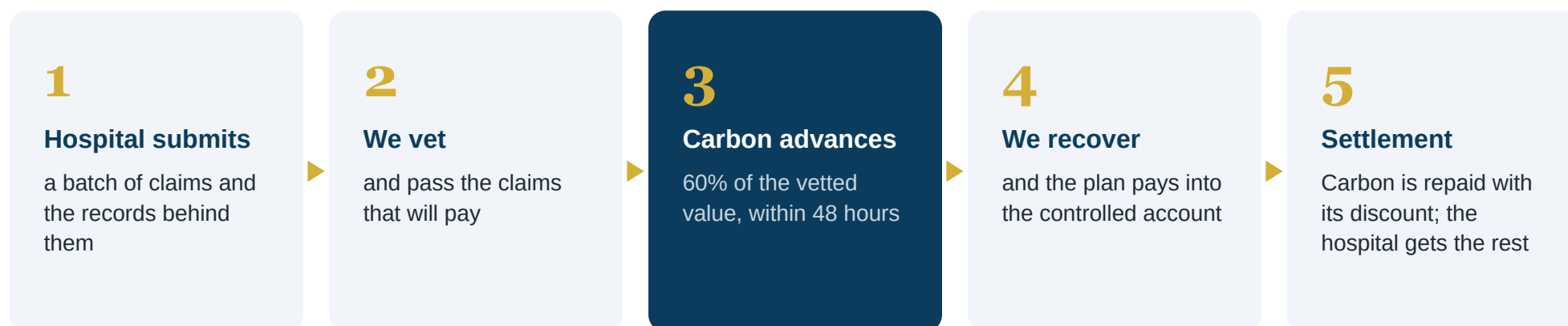
We match what the hospital billed against what each plan says it owes, and clear the gap claim by claim, with the plan, at the level where the decision is actually made.

Recover

We chase, escalate and settle, and payment goes to an account Carbon controls. Every claim is tracked from submission to cash, and the hospital sees the same status we do.



60% of a vetted claim, paid within 48 hours.



Sixty per cent is where we would open. The remaining 40% covers the rebilling at tariff, the odd claim that still fails after vetting, and Carbon's discount. Once a plan has paid on time for two or three cycles, its claims can earn a higher rate.



Carbon lends. We never hold the money or the credit risk.

Carbon	Consult for Africa	The hospital
Sets the credit policy and the facility limit for each hospital	Brings the hospitals, vets every claim before it is funded	Assigns the funded claims and tells the plans where to pay
Advances 60% of vetted value within 48 hours	Reconciles and recovers with the plans	Provides records, and repairs claims we send back
Holds the controlled account the plans pay into	Reports every claim's status to Carbon and the hospital	Repays any advance on a claim rejected for a reason it controlled
Prices the discount	Keeps the claims data clean enough to lend on	Keeps submitting through the desk, not around it



The vetting is the underwriting.

A claim is better collateral than most unsecured loans, because before funding it we can see who owes it, what was authorised and whether the paperwork will survive the plan's review. Losses come from funding claims that were never going to pay, and vetting is how those are kept out.

Control	How it works
Fund only what passes vetting	Authorisation, tariff and records checked before any advance
Cap the age of claims	Nothing older than an agreed age at submission is funded
Limit concentration	No single plan above an agreed share of any hospital's book
Control where the money lands	Plans told of the assignment and pay into Carbon's account
Recourse where the hospital caused the loss	Rejections for documentation or authorisation come back to the hospital
Start with hospitals we know	The first book comes from hospitals whose accounts we have already audited



Terms to settle before the first advance

Term	Our opening view
Advance rate	60% of vetted value, rising by payer once a plan pays reliably
Disbursement	Within 48 hours of a batch passing vetting
Discount	Carbon's to price, by days outstanding
Recourse	To the hospital for rejections within its control. Losses from a plan failing are a question for the pilot
Long stop	An unpaid advance falls due from the hospital at an agreed date
Collections	A controlled account at Carbon, with notice of assignment to each plan
Data	Claim-level performance shared with Carbon monthly, under a data sharing agreement that complies with the NDPA
Scope	Health plans and corporate accounts only. No collection from patients



A ninety day pilot with hospitals we have audited

- Three to five private hospitals in Lagos and Abuja, chosen from those we have already audited, so the first book is not a guess.
- A pilot facility sized by Carbon, with a limit per hospital and per plan.
- Measured on days from submission to cash, the share of disputes cleared, advances repaid on time, and losses.
- At day ninety, both sides decide on the advance rate, the recourse terms and whether to widen the facility, using the pilot's own numbers.

A small pilot should still produce Carbon's first claim-level default data on Nigerian health plans. There is very little of that anywhere.

From here to a first advance

- **This week.** A working session on the terms page, with whoever sets Carbon's credit policy.
- **Within a fortnight.** A shortlist of pilot hospitals, a sample of their claims for Carbon to test against its own model, and the data sharing agreement.
- **Within a month.** Signed pilot terms, and a first vetted batch.

Later, and separately: financing surgery for patients through the doctors in our network.

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